

# Where You Actually Feel It

## Structural Foundations of Health — Part 2 of 2

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### Last class, in one slide

**Without looking at your notes:** what does it mean that health is a *staircase* and not a *cliff*?

Turn to your neighbour. Thirty seconds.

Retrieval practice, not review. Take two answers, correct gently, move on. Target answer: health improves at every step up, so it is not just a poverty problem.

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### Today

Advantage is distributed unevenly, and the distribution has a history.

**Today: where you actually feel it.**

**Home**

**Work**

**Neighbourhood**

**Policy**

Then: **how you would study a real community** — and a group exercise doing it.

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## 1. Home

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## Housing does three separate things

- **What it is made of** — lead, mould, pests, cold
- **What it costs you** — rent takes the money that would have been food or a co-pay
- **Whether you get to stay** — three moves in a year: new school, lost doctor, no sleep

Only the first is about the building. The other two are **money** and **stability**.

Students default to “bad housing = mould”. Push them to the second and third, which do more damage and are less visible.

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## One concrete case: lead



Lead paint was legal in US homes until **1978** — and is still in those walls.

Old housing is concentrated in poor neighbourhoods. Lead damages the developing brain, and **the damage does not reverse**.

The cleanest single illustration of the whole course: a 1978 regulation, a 2026 child, and a mechanism you can draw on the board.

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## And the cost side

Paying more than **30%** of income on rent = **cost-burdened**. Above 50% = **severely** cost-burdened.

**Quick one:** rent goes up \$200 a month, income doesn't. What comes out of the budget first?

Name three things. Which of them are health?

Expected: groceries, prescriptions, the dentist, the car repair that gets you to work, heating. All of them are health. This makes housing cost a health variable, not a finance variable.

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## 2. Work

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### The wrong question and the right one

Wrong question: **do you have a job?**

Right question: **what does the job do to you?**

Two people both “employed” can have completely different health prospects.

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### Three ways a job reaches your body

The body takes it

Lifting, repetition, chemicals, heat



The mind takes it

High demand, low control, unpredictable hours



**It follows you home**

Pesticides, solvents, dust on your clothes

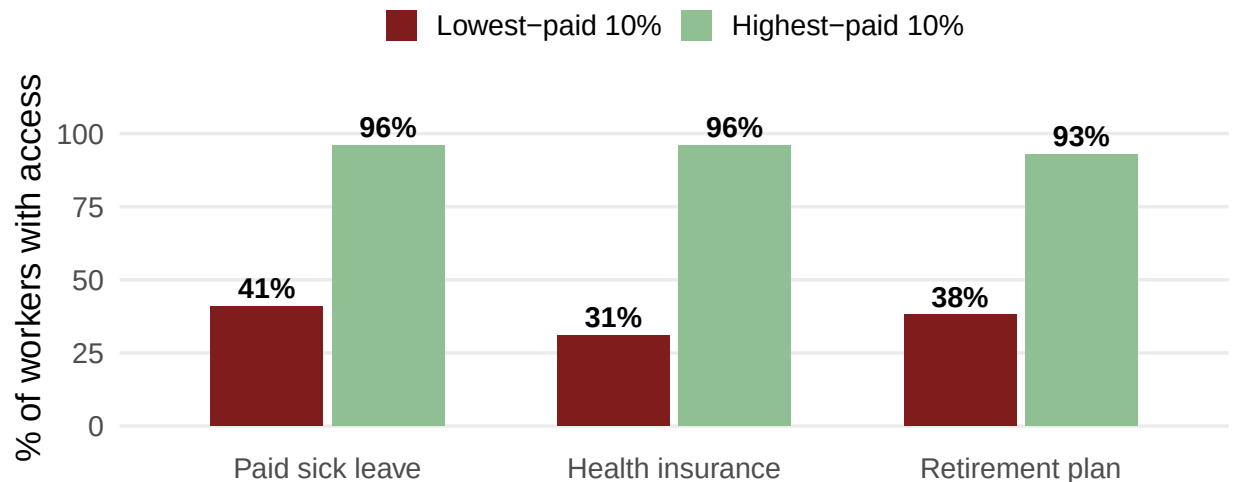


The middle one is the surprise: **high demand** + **low control** predicts heart disease. Being busy is not the problem. Being busy with no say is.

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## Who actually gets benefits

### Access to benefits, US private industry, March 2025



BLS, Employee Benefits in the United States, March 2025 (USDL-25-1464)

“Access” means the employer offers it, not that the worker takes it up — so these bars are the optimistic version.

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## Why paid sick leave is a health policy

**41%** of the lowest-paid workers can take a paid sick day. **96%** of the highest-paid can.

So the person handling your food, minding your child, or turning your grandmother in bed is the one who **cannot afford to stay home sick**.

Their working conditions are **your** exposure. This is why we study populations, not individuals.

This is the slide that makes the population-level argument land for students who came in thinking public health is about personal responsibility.

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## And a growing group with none of it

Independent contractors: **7.4%** of US employment — 11.9 million people.

Health insurance from *any* source:

- Traditional employees — **85%**

- Independent contractors — **74%**
- Temp agency workers — **61%**

No employer means no employer-sponsored anything: no sick leave, no workers' comp, no unemployment insurance. **Insecure work, insecure health.**

Source: BLS Contingent and Alternative Employment Arrangements, July 2023 (released Nov 2024). Note honestly that BLS does not publish an employer-coverage figure for contractors, because by definition there is no employer.

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### 3. Neighbourhood

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**What is built around you**



Ask what they can see: sidewalks, crossings, lighting, shade, benches, whether the shop is walkable. The point is that “exercise” is a design outcome, not only a willpower outcome.

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**And what is put near you**

**Environmental justice:** pollution, heat and industrial hazards are not spread evenly — they concentrate in low-income neighbourhoods and neighbourhoods of colour.

Remember last class: **cheap land is where the highway goes.**

**Which came first** — the polluting plant, or the poor neighbourhood around it?



There is evidence for both. Why does the answer matter for policy?

Genuinely contested in the literature (siting vs. subsequent demographic change). Use it to show that “which came first” changes whether the remedy is siting rules or cleanup and relocation.

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### What it does

- **Lungs** — asthma, COPD; children worst affected
- **Heart** — fine particulate exposure raises cardiovascular risk
- **Heat** — less shade, more asphalt; heatwave deaths cluster in the same blocks
- **Mind** — noise, no green space, and knowing this was allowed to happen here

This is the redlining map again, wearing a different coat.

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## 4. Policy

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### The lever

Home, work and neighbourhood are all **downstream of decisions somebody made**.

#### Pushes health up

- Income support (EITC)
- Medicaid expansion
- Paid leave laws
- Lead abatement funding

#### Pushes health down

- Zoning that blocks affordable housing
- Mass incarceration
- Weak housing-code enforcement

None of these is called a health policy. All of them are.

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## A natural experiment: Medicaid expansion

In 2014, some states expanded Medicaid and some did not. Same country, same decade.

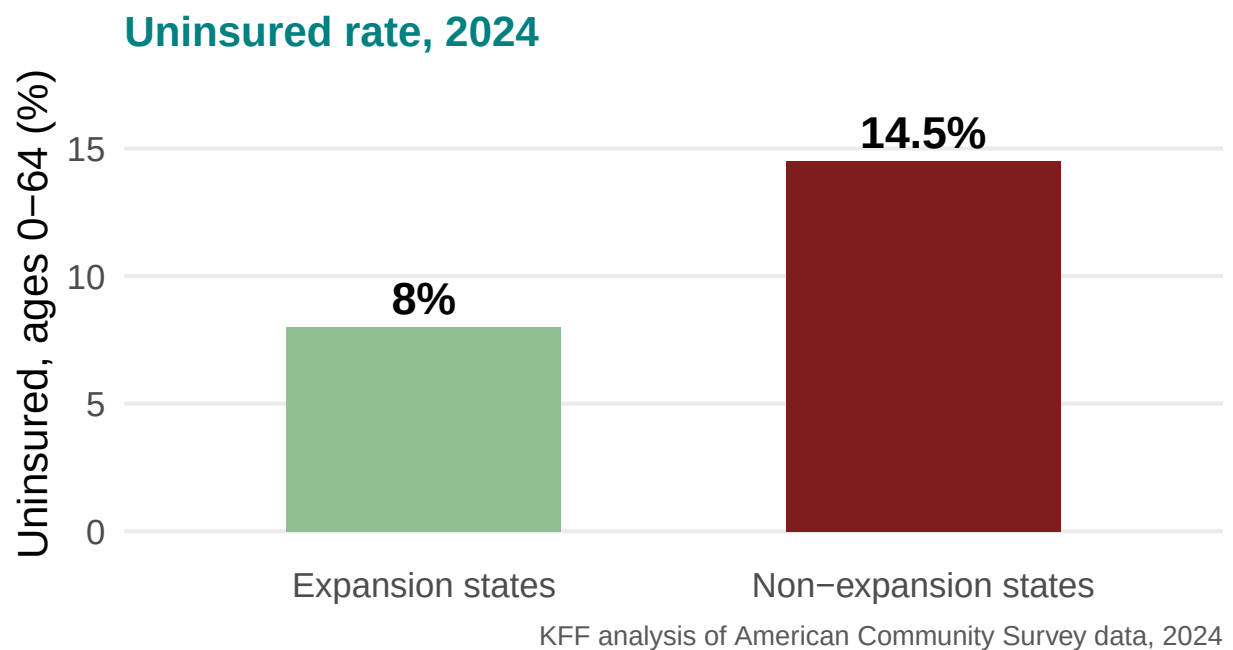
9.4%

Reduction in annual mortality among low-income adults aged 55–64 in expansion states.

Miller, Johnson & Wherry (2021), Quarterly Journal of Economics. Be precise about the population: low-income adults aged 55–64, not everybody. Rising to 11.9% by the third year. Roughly 15,600 deaths would have been averted had all states expanded.

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## And the rest of the picture



Expansion also cut medical debt sent to collections by about **42%** (Hu et al. 2018).

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## How would you study a community?

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## Two different jobs

### Community assessment

#### One place, in depth.

- You go there; you talk to people
- Assets as well as problems
- Ends with: a plan somebody can act on

### Population assessment

#### Many places, from above.

- Registries, surveys, vital statistics
- Compare counties, states, countries
- Ends with: a pattern, and where to look next

Neither is better. Each is bad at the other's question.

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## Same city, two questions

- **Population:** *Which PA counties have the highest diabetes rates?* → surveillance data, a map, a ranking
- **Community:** *Why this neighbourhood, and what already exists here that could help?* → walk the streets, count the shops, ask the clinic

**Your individual paper is a community assessment** — of your own hometown. That is why it cannot be written from data alone.